Triaperformance

TRIAPERFORMANCE GUIDE

Deciding at kilometre 30

The three goals, the procedure when the plan fails, and the sheet you carry with you

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Three sections from the Triaperformance Race Execution Guide: how to write your A, B and C goals before the race, exactly what to decide when the pace drops and the goal moves away, and the decision template that settles the day for you.

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Scope of this guide

This is training material. It is not medical or psychological advice.

It covers one thing, and it covers it completely: **how to execute a race, and what to do when the plan stops working.**

It does not cover anxiety or depression, it does not cover loss of motivation as a clinical matter, it does not cover exercise dependence, or your relationship with food or with your body. It also does not treat "mental toughness" as a character trait that needs fixing. Not because those things don't matter — they matter enormously — but because they are not my job. I'm your coach, not your psychologist, and that difference isn't a technicality: it's the line between what I can defend with my experience and what requires a licence.

On nutrition, this guide repeats nothing: everything on fuel, hydration and sodium lives in **The Fuel Kit**, and that's where you should look for it.

Talk to a professional before applying any of this if:

- You have an active injury, or pain a doctor hasn't looked at yet
- You have a diagnosed cardiovascular, respiratory or metabolic condition
- You are pregnant or trying to be
- You take medication regularly
- You are under 18

Talk to a mental health professional if:

This is not a diagnostic checklist. These are observations you can make about yourself, with a threshold of **time or function**, not of mood.

- The low after a race lasts **more than two weeks**, or stops you functioning at work, at home, or with the people around you
- You've lost interest in things that have nothing to do with sport
- Your sleep has come apart and isn't settling
- **You can't take a rest day**, or you train sick or injured because stopping feels impossible
- Your eating or your weight decides whether you train

If thoughts of harming yourself show up, that isn't an appointment you book for next week. Get help today.

What I do and what I don't

I adjust the plan, redistribute load to the sports that don't hurt, and hold the calendar with you. I don't diagnose, I don't assess injury risk, I don't give guidance on medication, and I don't coach weight. When something falls on that side of the line, I refer you out. That's the rule, and it has no exceptions.

> One more, and it organises everything that follows: **slowing down, walking and stopping are decisions with a cost, not failures of character.** This guide will never treat them as giving up.

A note from your coach

You train seriously. You do the intervals, you finish the long runs, you follow the data. You arrive on race day with the engine built.

And then the usual happens: the engine was fine and the race went badly.

For years this got explained with the word *mindset*. The athlete didn't have the head for it, wasn't strong enough, gave up. It's a convenient explanation and it's almost always wrong. What happens in a race isn't a collapse of character. **It's a series of decisions taken in the wrong order, on bad information, that were never once practised.**

The data is fairly clear about where it happens. In an analysis of over two million marathon records, the pacing collapse begins on average at **29.6 km** for men and **29.3 km** for women, and holds for the next ten kilometres. It happens to 28% of men and 17% of women. Not at the start, where everyone is focused. At kilometre 30, once you've already invested everything and the goal starts moving away from you.

That is the part of the race nobody trains you for.

This guide trains it. It won't tell you to be stronger. It gives you the written plan, the three goals, the pace ceiling for the opening kilometres, and — above all — **the procedure for deciding at kilometre 30**: continue, drop a goal, walk, or stop.

One rule organises everything else:

> **Nothing gets decided for the first time at kilometre 30.**

Everything you'll decide when you're empty, you already decided today, sitting down, with a clear head. On race day you don't think. You execute.

Let's get to work.

Iván
Founder & Head Coach, Triaperformance

1. Your three goals: A, B and C

This is the most important exercise in the guide. If you do only one, do this one.

Most athletes arrive at the start line with **one** goal. A number. And that number has a dangerous property: it's either met or broken. There's nothing in between.

When that number becomes impossible — and in one marathon out of four it does — the athlete is left without an objective at the worst possible moment. There's nothing to hold on to. That's where the race comes apart, and not for lack of legs: for lack of structure.

The correction is simple, and it has to be done before race week, in writing.

Goal A — the perfect day

The time you can run **if everything goes right**. Good weather, no wind, fuelling executed at 100%, no stomach problems, no cramps, two decent nights of sleep behind you.

Goal A is conditional, and it has to be written with its conditions attached. It isn't "3:15". It's: **"3:15, if the temperature is under 18°, if I take all six gels, and if I run the first half at 4:38."**

Once you write the conditions down, goal A stops being a promise and becomes a hypothesis. And a hypothesis can be discarded without the race collapsing on top of you.

Goal B — the normal day

The time you can run **on an ordinary day**. Something goes slightly wrong — something always does — and you still run well.

This is the real goal. It's the one that comes off most often, the one to train for, and the one that should govern your pace in the opening kilometres. The gap between A and B is usually 2 to 4%: on a three-and-a-half-hour marathon, four to eight minutes.

If, writing them down, you find A and B are thirty seconds apart, you don't have two goals. You have one with two names.

Goal C — the bad day

What you do when the day breaks. Extreme heat, a shut stomach, a cramp at 28 km, a pace that never shows up.

Goal C is not defined by a time. It's defined by a condition that depends only on you: *finish it walking and jogging, without injuring myself, and cross the line.* Or, in a race with cut-offs: *make the cut-off.*

Goal C is the most important of the three, because it's the only one still available when the other two are dead. An athlete with a written goal C doesn't have a ruined race. They have a different race.

The fourth line: when today isn't a race day

Under the three goals, write a fourth line. The conditions under which you **don't run**, or don't finish:

> *I don't start if I have a fever or an active infection.*
> *I stop if anything on the list in section 9 shows up.*

This isn't pessimism. It's what a pilot does before take-off: the abort criteria get defined on the ground, not in the air.

How to use them

- **Write them three to four weeks out**, after the simulation race (section 4), once you have real evidence of your pace.
- **Read them on race morning** — all three, out loud if it helps.
- **Drop one step at a time, never two.** A goes to B. B goes to C. An athlete who jumps from A straight to C at kilometre 30 is almost always quitting, not adjusting.

This isn't a mental technique. It's goal planning, exactly like periodising a block: you define the expected outcome, you define the conservative scenario, and you define the floor.

2. Kilometre 30: when the plan fails

The rest of the guide exists so that you can execute this section.

There's a moment in a lot of long races where this happens: the pace is dropping, the goal is moving away, and **you start doing arithmetic about quitting.**

That mental calculation — keep going or not, how far is left, what happens if I stop — is the moment this guide trains. It isn't weakness and it isn't a lack of character: it's what happens when you've invested heavily in a goal that has stopped being reachable. It happens to almost everyone, and it happens in roughly the same place.

What distinguishes an athlete who handles that stretch well isn't enduring more. **It's having a procedure.**

Recognising it

The signal is concrete: **you're calculating instead of running.**

When you catch yourself doing the arithmetic, don't keep doing it on autopilot. That's the point where you enter the procedure.

And the first move is to name the options, because there are three and not two:

1. **Hold the current goal**
2. **Drop one step of goal**
3. **Stop**

Most athletes in crisis only see the first and the third. That's the trap: it turns a race that could have been saved into a binary choice between suffering and quitting. **Option 2 exists, it's the right one nearly every time, and you have to name it out loud for it to show up.**

The three questions

Don't decide by feel. Decide with these three, in this order. They take about twenty seconds.

Question 1 — Is anything on the stop list happening?
Go to the list further down this section. If the answer is yes, there is no question 2. You stop.

Question 2 — Is there something that can be fixed?
Most kilometre-30 crises have a repairable cause, and the repair takes five to ten minutes to take effect:

- **When was your last gel?** If it's been more than thirty minutes, the crisis may simply be hunger. Eat something now and give it ten minutes before you decide anything.
- **When did you last drink?** Same.
- **Are you going faster than you think?** Look at the actual split, not the sensation.
- **Is there a cramp or a mechanical niggle?** Slow thirty seconds per kilometre for two kilometres and re-evaluate.

Don't make a big decision on an empty stomach. It's the most useful rule in this entire section. An enormous number of DNFs are hypoglycaemia misread as a failure of will.

Question 3 — Can I hold this pace to the next aid station?
Not to the finish. To the next aid station. If the answer is no, the pace is wrong and it has to come down — which, most of the time, means dropping a goal.

Dropping a goal

Dropping a goal is not giving up. It's the correct technical decision when the assumptions you built the objective on have changed. Exactly like adjusting the week's plan because you got sick.

How it's done, in order:

1. **You drop one step, not two.** A to B. B to C.
2. **You recalculate the pace** for the new objective. If you're on goal B with 12 km left, what pace do you need? That's your new number, and it's slower — that's the point.
3. **You let the old one go.** You stop looking at the splits for the goal you've just discarded. Continuing to compare against an objective that no longer exists is the fastest way to turn a bad day into a DNF.
4. **You re-segment.** You're no longer running 12 km. You're running to the next aid station. Win that stretch, then the next one.

Segmenting works **after** you've decided which goal you're chasing. Not before: "one kilometre at a time" without having resolved what you're chasing is just postponing the decision.

Walking

Walking isn't failing. Walking is a pacing tool, and most amateur athletes use it too late and use it badly.

What walking costs, with numbers. At 6:00/km, one minute of running gives you about 165 metres. One minute of fast walking gives you about 110. Walking that minute costs you roughly **twenty seconds**. Do the arithmetic at your own pace and you'll see it's almost always less than you fear.

What walking buys: a lower heart rate, a gel you can actually swallow, water that actually goes in rather than over your face, and — this is the important one — the ability to run again afterwards.

The two kinds of walking, and only one of them works:

Walking by decision	Walking by collapse
Chosen, with a limit set beforehand	It just happens, once there's no option left
"I walk the 30 seconds of the aid station"	"I don't know when I'll be able to run again"
You jog again at the point you defined	The walk extends itself
Costs twenty seconds	Costs twenty minutes

How to do it well:

- **Walk the aid stations.** All of them. From the first one, not from km 30. Thirty seconds walking lets you actually drink, and that's the point of the aid station.
- **If you have to walk outside an aid station, set a limit before you start:** to that post, to a count of sixty, to the bend. The limit gets defined **before** the first walking step, not during.
- **Run-walk is a strategy, not a defeat.** From km 32, alternating four minutes of jogging with one of walking is often faster — and considerably less miserable — than the continuous death march a lot of people end up in.

When to stop

This is the only part of the guide with no nuance, and where no goal matters.

Stop immediately and seek medical help if any of these appear:

- **Sharp, localised pain that changes how you land.** Not discomfort: pain that alters your mechanics.
- **Focal bone pain** — a specific point on a bone that hurts when you press it.
- **Chest pain, shortness of breath disproportionate to the effort, or irregular heartbeats.**
- **Confusion, disorientation, not understanding where you are or which kilometre it is.** This is the most dangerous one because it's the hardest to detect in yourself.
- **Dizziness with nausea, cold skin, or you've stopped sweating** in hot conditions.
- **Very dark urine together with severe muscle pain.**

On sodium and hydration problems — including hyponatraemia, which is serious and gets confused with dehydration — **The Fuel Kit** has the full section. I'm not repeating it here because that guide is the one that governs that topic.

Three rules around this list:

1. **No goal justifies ignoring it.** Not A, not C, not finishing. None.
2. **If you stop for anything on this list, you see a doctor.** Not "let's see how I feel tomorrow". That same day.
3. **If you can't assess yourself clearly, that is the answer.** Confusion doesn't self-diagnose. If you're doubting your own ability to judge, you already have the data.

And a general rule that applies to everything medical, in and out of a race: **I don't diagnose.** If any of this shows up, I refer you out and my part ends there. That's not legal caution — it's that it isn't my competence.

The cut-off

If your race has cut-offs, the arithmetic gets done **beforehand**, not while running.

What you need to know before you start:

- Which kilometre each cut-off is at
- The exact time it closes — normally clock time, not time since your own start
- Whether the clock runs from the gun or from when you crossed the mat. **This changes the calculation and a lot of people find out too late.**

The table you carry with you. For each cut-off, the average pace you need from wherever you are. Written down, laminated, or on the back of the bib:

> Cut-off at km 30, closes 14:30 · I started at 9:00 → I have 5:30 for 30 km → **11:00/km average**
> If at km 21 my watch says 3:10, I have 2:20 left for 9 km → **15:33/km**. I can walk it.

That second calculation is the one that saves races, because it turns a panic into a number. Plenty of athletes quit at a cut-off they were going to make comfortably, simply because they never did the sum.

The rule: if the number you need is slower than your fast walk, you carry on. If it's faster than your current pace with no margin, you drop to goal C and accept the cut-off as the day's objective.

3. Template — My decision playbook

Between six and ten lines. The trigger has to be something you can see or measure.

IF this happens (something I see or measure)	THEN I do this (one concrete action)

This is one part of the full guide

The complete Race Execution Guide adds the written race plan, the pace ceiling for the opening kilometres, where to put your attention by intensity, race week, the debrief and four printable templates. It is inside Triaperformance All-Access, along with the rest of the library.

See All-Access →

Would you rather we fitted it to your actual race? Write to me. coach@triaperformance.com · WhatsApp